



Business Plan

Claims and denial management for Florida physician practices

Half the industry price, month to month, in English, Spanish and Portuguese. Built from the ground up for Florida law, HIPAA and SOC 2.

Confidential. Prepared for prospective investors.

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Contents

1. Executive summary
2. The problem
3. The solution: the DenialDesk platform
4. Target market
5. Competitive positioning and brand
6. Business model and pricing
7. Go-to-market strategy
8. Financial plan
9. Product roadmap and milestones
10. Risks and mitigations
11. Use of funds and next steps

1 Executive summary

DenialDesk is a secure, cloud-based platform that helps Florida physician practices submit insurance claims, catch and work denials, enforce Florida's prompt-pay and appeal deadlines, and recover revenue they are legally owed.

Practices lose money to denied, underpaid and late-paid claims. Today they work those claims by hand across payer portals, spreadsheets and email, and they typically pay an outsourced billing or revenue-cycle vendor around **8% of revenue** on multi-year contracts with per-user license fees on top.

DenialDesk costs **4% of monthly revenue**. That is one flat, all-inclusive fee, with no per-license charges and no long-term contract, and support in **English, Spanish and Portuguese**.

4%

of monthly revenue, all-inclusive (industry: ~8%)

\$80K+

minimum annual savings for a \$2M practice

8–10

practices signed in Year 1

\$640–800K

annual run-rate at end of Year 1

Our promise to a physician: "At minimum, we will save you \$80,000 a year." We can show the number on one line of their own P&L, and it is the reason a practice switches.

2 The problem

For an independent physician practice, revenue depends on insurance claims being paid correctly and on time. It often isn't:

Denials are worked by hand

Staff move between payer portals, spreadsheets and email. The same denial types come back again and again, and no one fixes the root cause.

Deadlines are missed

Appeal and timely-filing windows run out silently. Once a deadline passes, that revenue is usually gone for good.

Florida law goes unenforced

Florida prompt-pay statutes (Fla. Stat. §§ 627.6131, 641.3155) require insurers and HMOs to pay on time or owe interest. Practices rarely track the clock, so they rarely collect.

No sense of what's worth chasing

Without ranking by dollar value and days left, staff work the oldest claim instead of the most valuable one.

And the vendors cost too much

Pain point	Industry norm
Expensive pricing	~8% of revenue
Long-term contracts	Multi-year lock-ins
Language barriers	English only, in a state where much of the workforce and patient base is Spanish- or Portuguese-speaking
Lack of transparency	Hidden fees, unclear pricing
Per-license fees	Charged per user or employee
High overhead	Heavy administrative and physician time spent on billing

3 The solution: the DenialDesk platform

DenialDesk is a multi-tenant web application. A practice's staff sign in and the platform covers the claim's whole life, from submission through payment, denial, appeal and outcome.

Capability	What it does for the practice
Practice setup	Locations, providers (NPI, taxonomy, Florida license) and payers, each tagged with its legal regime: Florida insurer, Florida HMO, ERISA self-funded, Medicare or Medicare Advantage.
Claim submission	Sends electronic claims (X12 837P) through a contracted clearinghouse after a timely-filing check, and captures acknowledgments (999, 277CA).
Prompt-pay enforcement	Starts the Florida prompt-pay clock on receipt, sends alerts, produces late-payment interest worksheets and flags uncontestable obligations.
Remittance posting	Imports payer remittances (835), checks balances, and captures every denial with its standard reason codes (CARC / RARC / group code).
Denial work queues	Ranks denials by dollar value and days left to the appeal deadline, so staff work the most valuable claim first.
Appeals	Tracks each appeal from draft to decision, with deadlines calculated by the rules engine (never guessed). Recovered amounts feed back into reporting.
Evidence and audit	Keeps an immutable audit log and claim version history, and exports evidence for complaints to Florida's Office of Insurance Regulation.
Reporting	Standard revenue-cycle reports on denial rate, A/R, recovery and staff workload.

What sets the technology apart

- **Florida rules engine.** Every statutory deadline, interest rate and threshold is a versioned, effective-dated rule with its legal citation, with business-day and holiday calendars in Eastern time. When the law changes, we update one rule instead of rewriting code.
- **Security and compliance from day one.** Single sign-on with MFA, row-level tenant isolation in the database, AES-256 encryption at rest with field-level encryption for sensitive identifiers, and full audit logging. The platform targets HIPAA, OWASP ASVS Level 2 and a SOC 2 Type I audit.
- **U.S.-only data residency.** Florida law (Fla. Stat. § 408.051(3)) bans offshore storage of patient data. Production runs on Microsoft Azure in U.S. regions only.
- **No code inflation.** The system never changes a CPT, ICD or HCPCS code without a recorded human approval. This protects practices from fraud-and-abuse exposure.

Where the build is today

The foundation is complete: the application shell, authentication with MFA, tenant isolation with automated tests, the audit log, the rules engine, a synthetic-data generator, and per-practice BAA tracking. MVP features are well underway: claims, the denial queue, payer catalog, remittance (835) import and posting, prompt-pay clocks with interest worksheets, and the first stage of appeals. Development uses synthetic data only and runs on a pre-production environment.

4 Target market

Primary: mid-market physician practices

Independent Florida practices with around **\$2M in annual revenue**. They are big enough to feel the 8% vendor fee (about \$160K a year) but too small for an in-house revenue-cycle department.

Secondary: startup practices

New practices choosing their first billing platform. They have no contract to break, are price-sensitive, and value month-to-month terms.

Why Florida first

- Florida has a distinctive and strict legal framework (prompt pay, offshore-data ban, patient refund rules). That creates demand for a specialist and a barrier to generic national tools.
- Florida's large Spanish- and Portuguese-speaking communities are underserved by English-only vendors. Our multilingual support is a real reason to choose us, not a nice extra.
- Payer mix at launch: Florida-regulated commercial insurers, Florida HMOs, Medicare Part B (First Coast) and Medicare Advantage. Florida Medicaid follows in Phase 2.

Who uses it inside the practice

User	What they need
Billers / denial specialist	Submits claims and works denial and follow-up queues day to day
Revenue-cycle manager	Watches A/R, denial rate, recovery and staff workload
Practice administrator	Sets up the practice, providers, payers and users
Compliance officer	Reviews audit logs, refunds and evidence of payer violations
Physician / owner	Sees lower cost and recovered revenue

5 Competitive positioning and brand

Challenge	The industry norm	The DenialDesk solution
Expensive pricing	~8% of revenue	4% of monthly revenue , half the competition's cost
Long-term contracts	Multi-year lock-ins	Month to month . No long-term commitment
Language barriers	English only	English, Spanish and Portuguese
Lack of transparency	Hidden fees	Upfront pricing . Nothing hidden
Per-license fees	Charged per user	No per-license charges . One flat fee
High overhead	Heavy admin and physician time	Aim: cut admin and physician billing time by more than half

Brand pillars

1 • Affordable

Half the industry rate.

2 • Flexible

Month to month. We keep clients by earning it.

3 • Multilingual

English, Spanish, Portuguese.

4 • Transparent

4% of monthly revenue, stated upfront.

5 • Simple

One flat fee. No per-license charges.

6 • Proven value

At least \$80K a year in savings for a \$2M practice.

Defensibility

- **Florida-specific depth:** encoded statutes, deadlines and interest rules that generic national tools don't carry.
- **Compliance posture:** SOC 2, HIPAA Business Associate controls and U.S.-only residency are costly and slow for a new competitor to copy.
- **Cost structure:** software-led workflows let us profitably charge half the market rate.

6 Business model and pricing

- **4% of monthly revenue:** flat and all-inclusive, billed monthly.
- **No per-license or per-employee fees:** the practice can add all its staff at no extra cost.
- **Month to month:** no long-term contract.
- **Fully transparent:** the price is the price.

Value to the customer: a \$2M practice

	Competitor (8%)	DenialDesk (4%)
Annual fee on \$2M revenue	\$160,000	\$80,000
Per-employee license fees	Varies	\$0
Annual savings		≥ \$80,000

Total savings are at least \$80,000, plus whatever the practice spends today on per-employee licenses. The more they spend on licenses now, the more they save with us.

"At minimum, we will save you \$80,000."

Unit economics per client

Metric	Value
Annual revenue per \$2M client (4%)	\$80,000
Monthly revenue per client	≈ \$6,667
Target gross margin (software-led service)*	65–75%
Target customer acquisition cost*	≤ \$15,000
CAC payback at 70% margin*	≈ 3–4 months

* Planning targets, to be confirmed against actual clearinghouse, hosting and staffing costs.

7 Go-to-market strategy

Phase A: design partners (Months 1–6)

- Sign 2–3 founding practices in South and Central Florida at the standard 4%, with priority onboarding and direct input into the roadmap.
- Measure savings, recovered revenue and time saved, and turn them into case studies in all three languages.

Phase B: founder-led sales (Months 4–12)

- Direct outreach to practice owners and office managers with a **free savings analysis**: we compare their current vendor invoice with 4% and show the \$80K+ difference.
- Target practices at contract renewal, when switching costs are lowest.
- Bilingual sales and onboarding (English, Spanish, Portuguese) for Hispanic- and Brazilian-owned practices in Miami-Dade, Broward, Palm Beach and Orange counties.

Phase C: repeatable channels (Year 2+)

- **Referral program**: credits for practices that refer peers.
- **Partners**: practice-management consultants, healthcare CPAs, medical associations and county medical societies.
- **Startup practices**: partnerships with healthcare attorneys and lenders who advise physicians opening a practice.
- **Content**: Florida prompt-pay and denial guides in three languages, plus webinars.

Onboarding and retention

Because we are month to month, we have to earn every renewal. Each client gets a monthly value report showing dollars recovered, interest collected, denials overturned and fees saved compared with their previous vendor.

Sales funnel to reach 10 practices in Year 1 (planning assumptions)

Stage	Count	Conversion
Practices contacted	250	
Savings analyses delivered	60	24%
Proposals	25	42%
Signed practices	10	40%

8 Financial plan

Year 1 revenue target

Practices signed	Annual revenue per client	Full-year run-rate
8	\$80,000	\$640,000
10	\$80,000	\$800,000

Run-rate figures. Revenue actually recognized in Year 1 depends on when each practice signs.

Year 1 recognized revenue (signing ramp)

If we sign one practice a month from Month 3 through Month 12 (10 practices), active client-months total 55. Year 1 **recognized revenue is about \$367,000**, and we exit the year at an **\$800,000 annual run-rate**.

Three-year projection (base case)

	Year 1	Year 2	Year 3
Practices at year end	10	30	65
Average active practices	4.6	20	47
Revenue	\$367K	\$1.60M	\$3.76M
Exit annual run-rate	\$800K	\$2.40M	\$5.20M
Cost of service (clearinghouse, hosting, support)	\$150K	\$480K	\$1.03M
Gross profit	\$217K	\$1.12M	\$2.73M
Operating expenses (engineering, sales, compliance, G&A)	\$1.10M	\$1.50M	\$2.10M
Operating income (loss)	(\$883K)	(\$380K)	\$630K



Base case assumes every client is a \$2M-revenue practice paying 4%, no churn in Year 1 and 5% annual churn after that. Costs are planning estimates. Year 3 is the target break-even year. All projections are forward-looking and will be updated as real client data comes in.

Scenarios: year-end practices

Scenario	Year 1	Year 2	Year 3	Year 3 exit run-rate
Conservative	8	20	40	\$3.2M
Base	10	30	65	\$5.2M
Upside	12	40	90	\$7.2M

9 Product roadmap and milestones

Phase	Scope	Status
Phase 0: Foundation	Tech stack, app shell and design system, SSO + MFA, tenant isolation, audit log, rules engine, synthetic data, BAA tracking	Largely done
Phase 1: MVP	Florida commercial, Florida HMO, Medicare and Medicare Advantage: claim submission (837P), acknowledgments, prompt-pay tracking, remittance posting (835), denial queues, appeals, evidence export, SOC 2 Type I	In progress
Phase 2	Florida Medicaid (FFS + SMMC), self-funded ERISA workflows, AHCA dispute packets, patient refund tracker	Planned
Phase 3	AI denial prediction and AI-drafted appeals (human-reviewed), FHIR prior-authorization API, workers' comp / PIP, patient payments	Planned

Milestones before the first live client

- Florida healthcare counsel engaged and regulatory-role memo signed off
- Azure Business Associate Agreement in place; clearinghouse selected under a subcontractor BAA
- Security Officer and Privacy Officer named; risk analysis and core policies
- Production security gates: passkeys and just-in-time privileged access for operators, break-glass alerting
- SOC 2 Type I audit

10 Risks and mitigations

Risk	Mitigation
Regulatory and legal interpretation	All deadlines and rates live in a versioned rules engine and are flagged for verification until Florida healthcare counsel confirms them. The percentage-of-revenue fee structure will be reviewed by counsel for Medicare reassignment and fee-splitting rules.
Data breach / HIPAA	Encryption, tenant isolation, MFA, immutable audit log, U.S.-only hosting, SOC 2 program and cyber insurance.
Sales cycle longer than planned	Savings-analysis offer, renewal-timed outreach, and design partners who become references.
Churn (month-to-month)	Monthly value reports, multilingual support, and measurable savings.
Clearinghouse dependency	Integration kept behind an adapter so we can switch or add a second clearinghouse.
Margin pressure at 4%	Software automation on queues, deadlines and appeals keeps cost per claim low. Gross margin is tracked monthly.
Incumbent price response	Florida depth, language coverage and no lock-in are harder to match than a price cut.

Forward-looking statements. This plan contains projections and planning assumptions that involve risk and uncertainty. Actual results may differ materially. Savings figures assume a \$2M practice currently paying about 8% of revenue.

11 Use of funds and next steps

We are raising [amount to be confirmed] to complete the MVP, reach SOC 2 Type I, and sign our first 10 practices.

Use	Share	Purpose
Product and engineering	45%	Finish the MVP, clearinghouse integration and Phase 2 groundwork
Sales and marketing	25%	Bilingual sales, savings analyses, content in three languages
Compliance and security	15%	SOC 2 audit, counsel, security tooling, insurance
Customer onboarding and support	10%	Multilingual onboarding and billing specialists
Working capital	5%	Runway buffer

Next 12 months

- **Q1:** Counsel sign-off, clearinghouse contracted, MVP feature-complete, first design partner onboarded
- **Q2:** SOC 2 Type I audit; 3 live practices
- **Q3:** Referral and partner channels launched; 6 practices
- **Q4:** 8–10 practices; \$640K–\$800K annual run-rate

DenialDesk is affordable, flexible, multilingual, transparent and simple, with proven value. Florida practices keep more of what they earn.

